

David W. Bynon

January 3, 2026

To:

The Honorable Jim O'Neill, Deputy Secretary of Health and Human Services

Dr. Stella Mandl, Director, Office of Healthcare Experience and Interoperability (OHEI)

Assistant Secretary for Technology Policy (ASTP/ONC)

Re:

Submission to Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

(Docket ID: HHS-ASTP-2025-23641)

Subject: A Scalable Protocol for Deflationary AI Governance and Program Integrity

Dear Deputy Secretary O'Neill and Dr. Mandl,

I am submitting the enclosed comment in response to HHS's Request for Information on accelerating the adoption and use of artificial intelligence in clinical care. The core challenge HHS faces is not whether AI can be deployed, but how to scale its use without expanding federal operational burden or relying on opaque, non-scalable model-level oversight.

The enclosed comment describes a scalable, non-device AI governance protocol that enables HHS to accelerate adoption by aligning semantic responsibility with carriers, providers, and intermediaries—where existing accuracy obligations already reside—while preserving federal oversight through deterministic, audit-ready outputs. This approach operationalizes the “AI-enabling infrastructure and documentation” principles articulated in OMB Memorandum M-25-21 by making authoritative policy meaning explicit, versioned, and auditable for high-impact administrative AI.

Rather than relying on probabilistic inference from narrative text, this protocol requires AI systems to retrieve from authoritative, machine-readable policy memory. Doing so enables AI outputs to be verified directly against versioned, cryptographically anchored definitions without inspecting model internals or reconstructing inference. The result is audit-by-design oversight that reduces downstream reconciliation costs currently borne by CMS, OMHA, and beneficiary support operations, while allowing industry-led deployment of AI-assisted care and benefits operations without introducing model-specific regulatory regimes or expanding federal operational scope.

If useful, I would welcome the opportunity to brief the HHS AI Governance Board or CMMI staff on how this protocol could be explored through existing sub-regulatory guidance or voluntary innovation pilots.

Respectfully,

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